

United for Global Mental Health, London E1 6RA, UK (AM); Keri: Caring for Activists, Davao, Philippines (NA); School of Population and Global Health, McGill University, Montreal, QC, Canada (JA); MHPSS Collaborative, Hosted by Save the Children Denmark, Copenhagen, Denmark (JA, VU); The George Institute for Global Health, London, UK (CSB); Global Climate and Health Alliance, London, UK (JB); Soulbeegood, São Paulo, Brazil (LRC); Vertentes Mental Health Ecosystem, São Paulo, Brazil (LRC, JF); The George Institute for Global Health, New Delhi, India (MD); ASEc+, São Paulo, Brazil (JF); Columbia University Mailman School of Public Health, New York, NY, USA (NG); AzerMDS, Baku, Azerbaijan (NG); UNICEF, New York, NY, USA (ZH); The Institute of Global Health Innovation, Imperial College London, London, UK (ELL, OeO); Grantham Institute for Climate Change and the Environment, Imperial College London, London, UK (ELL, OeO); The Blavatnik School of Government, University of Oxford, Oxford, UK (OeO); Mohammed V University in Rabat, Rabat, Morocco (SU); Bangalore Medical College and Research Institute, Bangalore, India (HU); Health Working Group of YOUNGO, UNFCCC, Bonn, Germany (HU)

- 1 Lawrance EL, Thompson R, Newberry Le Vay J, Page L, Jennings N. The impact of climate change on mental health and emotional wellbeing: a narrative review of current evidence, and its implications. *Int Rev Psychiatry Abingdon Engl* 2022; **34**: 443–98.
- 2 Corvalan C, Gray B, Villalobos Prats E, Sena A, Hanna F, Campbell-Lendrum D. Mental health and the global climate crisis. *Epidemiol Psychiatr Sci* 2022; **31**: e86.
- 3 Sindall C, Lo S, Capon T. Governance for the well-being of future generations. *J Paediatr Child Health* 2021; **57**: 1749–53.

- 4 Massazza A. COP29: how climate stressors are impacting mental health in Azerbaijan, and what we can do about it. 2024. <https://unitedgmh.org/the-global-advocate/cop29-how-climate-stressors-are-impacting-mental-health-in-azerbaijan-and-what-we-can-do-about-it/> (accessed Nov 1, 2024).
- 5 UAE Consensus, Conference of Parties 28. COP28 declaration on climate and health. 2023. <https://www.cop28.com/en/cop28-uae-declaration-on-climate-and-health> (accessed Nov 11, 2024).
- 6 Global Mental Health Action Network. Advocacy briefs: climate change, environment, mental health and wellbeing – impacts and practical solutions. March 18, 2024. <https://gmhan.org/news/mh-environment-advocacy-briefs> (accessed Nov 11, 2024).
- 7 WHO. Communicating on climate change and health: toolkit for health professionals. March 22, 2024. <https://www.who.int/publications/i/item/9789240090224> (accessed Nov 11, 2024).
- 8 Massazza A. Mental health at COP28: what's next? 2023. <https://unitedgmh.org/the-global-advocate/mental-health-at-cop28-what-next/> (accessed Nov 1, 2024).
- 9 WHO. Climate and health resolution. June 1, 2024. https://apps.who.int/gb/ebwha/pdf_files/WHA77/A77_R14-en.pdf (accessed Nov 11, 2024).
- 10 WHO. 2023 WHO review of health in nationally determined contributions and long-term strategies: health at the heart of the Paris Agreement. Nov 22, 2023. <https://www.who.int/publications/i/item/9789240074729> (accessed Nov 11, 2024).

Global action on problematic usage of the internet: announcing a *Lancet Psychiatry* Commission



Digital technologies and societal problems are increasingly interconnected, and this nexus has important implications for mental health globally.^{1,2} Concerns exist about problematic usage of the internet (PUI), digital technology-related disorders, and their public health and societal costs.³ One 2020 meta-analysis found that around 7% of the world's population show signs of internet addiction, and prevalence estimates have shown an increase over time, especially among young people.⁴ Specific types of PUI, such as problematic smartphone use, might have an even higher prevalence.⁵ Increased awareness of the association between PUI and mental disorders in general and the potential negative effects of social media use in particular has led to calls for more action to protect individuals, particularly children and youth, from online harm.^{6,7}

Specific types of PUI have been recognised as mental and behavioural disorders in the ICD-11. These include online forms of gambling disorder and gaming disorder, which are classified primarily as disorders due to addictive behaviours and secondarily as impulse control disorders, and compulsive sexual behaviour disorder, which is classified as an impulse

control disorder and often expressed online. Other digital technology-related behaviours might be considered as other specified disorders due to addictive behaviours, including compulsive buying or shopping behaviours and problematic usage of social media.⁸ Important advances have been made in developing clinical measures for these conditions, and research is accumulating on the overlapping and distinctive psychobiology, psychotherapy, pharmacotherapy, and neuromodulation related to specific types of PUI.³

Despite important efforts, much more work is needed from multiple stakeholders in clinical, public health, governmental, and other sectors.⁹ Key questions for society to address include the biological, psychological, social, and commercial risk and resilience factors that lead to or mitigate against PUI, the individual and public health consequences of PUI, and the nature and outcomes of health systems and services for individuals with PUI. A key area is the relevance to PUI of regulatory frameworks related to substances, gambling, and harmful consumer products: aspects of these regulatory frameworks might be applicable to PUI, but several unique aspects of PUI seem to require specific approaches. Particular concerns are the ready access of

Published Online
October 17, 2024
[https://doi.org/10.1016/S2215-0366\(24\)00323-7](https://doi.org/10.1016/S2215-0366(24)00323-7)

minors to the internet, the use of addictive design and manipulative algorithms by technology companies to encourage and direct online attention, and the ever-increasing power of artificial intelligence (AI) to strengthen such designs and algorithms.

The *Lancet Psychiatry* Commission on global action on problematic usage of the internet aims to provide evidence-based recommendations to improve health practices and advance health policy globally in this crucial area. The Commission will assemble a multidisciplinary and international group of authors—with expertise in compulsive, impulsive, and addictive behaviours, public health and policy—as well as an advisory group including people with lived experience of being negatively affected by PUI. The confluence of recent investment in networked research to detect risk and prevent PUI in teenagers at an international level,¹⁰ advances in large language models, and ongoing legislative attempts to address online safety and to regulate AI in the USA, Europe, and Asia make this effort timely.

The ultimate goal of the Commission is to set a clear agenda to guide the development and provision of evidence-based health and social care for those affected by digital harms, and to guide action to reduce population-level digital technology-related harms through public health, regulatory measures, and preventive or therapeutic interventions. The Commission aims to summarise knowledge in this area, consider diagnostic and assessment challenges, encourage implementation of evidence-based interventions and regulations, and make research recommendations to address gaps in our knowledge and practices. We are particularly concerned about vulnerable groups, such as developing young people. The Commission welcomes written input from clinicians and policy makers with an interest in this area, as well as from those with lived experience of PUI. Through the involvement of a diverse group of stakeholders, the Commission aims to provide a framework to limit potential harms while simultaneously affording civil liberties and harnessing positive aspects of digital technologies.

NAF has received support for research or networking from the UK National Institute for Health and Care Research, UK Research and Innovation, COST Action, Orchard, and Compass Pathways; accepted travel or hospitality expenses from the British Association for Psychopharmacology, European College of Neuropsychopharmacology, Royal College of Psychiatrists, International College

of Neuropsychopharmacology, International Forum of Mood and Anxiety Disorders, and World Psychiatric Association; has received payment from Elsevier for editorial duties and the Mental Health Academy for lecturing; has accepted paid speaking engagements in various industry-supported symposia; and recruited patients for various industry-sponsored studies in the field of OCD treatment. NAF also leads an NHS treatment service for OCD; holds board membership for various registered charities linked to OCD; gives expert advice on psychopharmacology to the UK Medicines and Healthcare products Regulatory Agency; and has participated in a WHO working group focusing on diagnosis and classification of obsessive compulsive or related disorders for the ICD-11. ZD reports that the University of Gibraltar received funding from the Gibraltar Gambling Care Foundation (an independent, not-for-profit charity) and donations from gambling operators through the Licence Conditions and Codes of Practice RET process supervised by the UK Gambling Commission. MNP has consulted for Opiant Therapeutics, Game Day Data, Baria-Tek, and Boehringer Ingelheim Pharmaceuticals; has been involved in a patent application with Yale University and Novartis; has received research support from Mohegan Sun Casino, Children and Screens, and the Connecticut Council on Problem Gambling; has participated in surveys, mailings, or telephone consultations related to drug addiction, impulse-control disorders or other health topics; has consulted for or advised gambling, non-profit, and legal entities on issues related to impulse control, internet use, and addictive disorders; has performed grant reviews for research-funding agencies; has edited journals and journal sections; has given academic lectures in grand rounds, Continuing Medical Education events, and other clinical or scientific venues; and has generated books or book chapters for publishers of mental health texts. HE has received funding from the Medical Discovery Team on Addiction, University of Minnesota, William K Warren Foundation, Laureate Institute for Brain Research, and Brain and Behavior Foundation. HB-J is director of the National Centre for Behavioural Addictions in the UK, vice-president of the Royal Society of Medicine, and national advisor to Gambling Harms for England. BK is chair of the 5Rights Foundation and chair of Digital Futures for Children. DJS has received consultancy honoraria from Discovery Vitality, Johnson & Johnson, Kanna, L'Oreal, Lundbeck, Orion, Sanofi, Servier, Takeda, and Vistagen. All other authors declare no competing interests.

Naomi A Fineberg, Zolt Demetrovics, Marc N Potenza, Gemma Mestre-Bach, Hamed Ekhtiari, Andres Roman-Urrestarazu, Sophia Achab, Thomas Kattau, Henrietta Bowden-Jones, Shane A Thomas, Thomas F Babor, Beeban Kidron, *Dan J Stein
dan.stein@uct.ac.za

Hertfordshire Partnership University NHS Foundation Trust, Hertfordshire, UK (NAF); School of Life and Medical Sciences, University of Hertfordshire, Hatfield, UK (NAF); School of Clinical Medicine, University of Cambridge, Cambridge, UK (NAF); College of Education, Psychology and Social Work, Flinders University, Adelaide, Australia (ZD); Institute of Psychology, ELTE Eötvös Loránd University, Budapest, Hungary (ZD); Centre of Excellence in Responsible Gaming, University of Gibraltar, Gibraltar (ZD); Department of Psychiatry and Child Study Center, Yale School of Medicine, New Haven, CT, USA (MNP); Connecticut Mental Health Center, New Haven, CT, USA (MNP); Connecticut Council on Problem Gambling, Wethersfield, CT, USA (MNP); Department of Neuroscience and Wu Tsai Institute, Yale University, New Haven, CT, USA (MNP); Instituto de Investigación, Transferencia e Innovación, Universidad Internacional de La Rioja, La Rioja, Spain (GM-B); Department of Psychiatry and Behavioral Sciences, University of Minnesota, Minneapolis, MN, USA (HE); Mental Health Policy Economics Group, Department of Psychiatry, University of Cambridge, Cambridge, UK (AR-U); Sección de Psiquiatría del Niño y del Adolescente, Pontificia Universidad Católica de Chile, Santiago, Chile (AR-U); Bennett Institute for Public Policy, University of Cambridge, Cambridge, UK (AR-U); ReConnecte Specialized Centre, Addiction Division, University Hospitals of Geneva, Geneva, Switzerland (SA); WHO Collaborating Center, Department of Psychiatry, Faculty of Medicine, Geneva University, Geneva, Switzerland (SA); Advisory Board on Internet-related disorders, Swiss Federal Office of Public Health, Bern, Switzerland (SA); International Drug Policy Academy, Pompidou Group, Strasbourg, France (TK); National Centre for Behavioural Addictions, London, UK (HB-J); Department of Psychiatry, University of Cambridge, Cambridge, UK (HB-J); Faculty of Brain Sciences, University College London, London, UK (HB-J); Institute of Health and Wellbeing, Federation University of Australia, Ballarat, VIC, Australia

(SAT); Department of Public Health Sciences, University of Connecticut School of Medicine, Farmington, CT, United States (TFB); 5Rights Foundation, London, UK (BK); SAMRC Unit on Risk & Resilience in Mental Disorders, Department of Psychiatry & Neuroscience Institute, University of Cape Town, Cape Town 7925, South Africa (DJS)

- 1 Stein DJ, Chamberlain SR, Fineberg NA, eds. *Mental health in a digital world*. Elsevier, 2022.
- 2 Potenza MN, Faust KA, Faust D, eds. *The Oxford handbook of digital technologies and mental health*. Oxford University Press, 2020.
- 3 Fineberg NA, Menchón JM, Hall N, et al. Advances in problematic usage of the internet research—a narrative review by experts from the European network for problematic usage of the internet. *Compr Psychiatry* 2022; **118**: 152346.
- 4 Pan Y-C, Chiu Y-C, Lin Y-H. Systematic review and meta-analysis of epidemiology of internet addiction. *Neurosci Biobehav Rev* 2020; **118**: 612–22.
- 5 Meng S-Q, Cheng J-L, Li Y-Y, et al. Global prevalence of digital addiction in general population: a systematic review and meta-analysis. *Clin Psychol Rev* 2022; **92**: 102128.
- 6 Hartford A, Stein DJ. Attentional harms and digital inequalities. *JMIR Ment Health* 2022; **9**: e30838.
- 7 The US Surgeon General's Advisory. Social media and youth mental health. 2023. <https://www.hhs.gov/sites/default/files/sg-youth-mental-health-social-media-advisory.pdf> (accessed Oct 10, 2024).
- 8 Brand M, Rumpf H-Jü, Demetrovics Z, et al. Which conditions should be considered as disorders in the International Classification of Diseases (ICD-11) designation of "other specified disorders due to addictive behaviors"? *Journal of Behavioral Addictions* 2020; **11**: 150–59.
- 9 Swanton TB, Blaszczyński A, Forlini C, Starcevic V, Gainsbury SM. Problematic risk-taking involving emerging technologies: a stakeholder framework to minimize harms. *J Behav Addict* 2021; **9**: 869–75.
- 10 Community Research and Development Service, EU Commission. Boosting societal adaptation and mental health in a rapidly digitalizing, post-pandemic Europe. April 21, 2023. <https://cordis.europa.eu/project/id/101080238> (accessed Oct 10, 2024).